



Nursing Practice Guideline

Eye Treatments

Reference #: NPG21

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation-Wide	Nursing

Introduction

This guideline outlines the procedures and responsibilities of individuals and the health service organisation in relation to Eye Treatments. It ensures the delivery of a uniform, integrated multidisciplinary approach with management that is consistent with best practice.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☒	Misconduct	☐
Breach SCGH Mission & Values	☒	Staff Safety	☒
Other:	☐		

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1.1 Medical Requirements

The Medical Officer is to prescribe eye drops and / or ointment for the Ophthalmological patient:

- SCGH Eye Treatment Chart,
- OPH Pre-Operative Eye Chart or the WA In-Hospital Medication Chart.

1.2 Safety Information

Ensure contact lenses are removed prior to medication administration.

Pressure must never be placed on an eyeball following surgery to the globe.

Eye medications are single patient use. Eye medications must be discarded when no longer required by the patient.

Eye drops should be checked with another nurse at the beginning of each shift.

Punctal pressure (application of a finger gently against the tear duct in the inner corner of the eyelid) is to be used when administration of eye drops occurs; to avoid systemic absorption.

Eye drops may affect patient's vision and may increase risk of falls.

Patients should be assessed and screened to identify risk of fall using the Falls Risk Management Tool, and include fall prevention strategies.

Use of appropriate PPE as per standard precautions

If gloves are required, ensure that the non-sterile gloves are clean and free of visual contaminants, including powder.

Contact lens is to be removed in presence of foreign bodies, chemical irritants, and if patient has altered level of consciousness.

1.3 Nursing Management

- All medications must be administered as per Nursing Practice Guideline No 51- Drug Administration.
- All prescribed eye medications are to be stored on a disposable tray at the patient bedside locker. Refrigeration of eye medication is not required once opened, with the exception of preservative free medications, which must be kept on ice at patient's bedside.
- One eye toilet pack is to be used per eye to prevent transmission of microorganisms.^{1 Level 5}
- When the patient is being acutely treated under the direction of an ophthalmologist and there is infection present in the eye, separate eye drops should be used for each eye; eye drop medication bottles should be labelled left and right.
- Pre-medication eye drops must accompany the patient to theatre. If not required in post-operative treatment, the pre-medication eye drops are to be disposed of on return to the ward.
- Eye drops / ointment must be identified and labelled with the patient's name and opening date. Eye drops / ointment are to be disposed of after **one month**, unless otherwise indicated.

- Ensure contact lenses and prosthesis are placed in a solution of sodium chloride 0.9% until the lens can be returned to the patient, or patient can resume own lens prosthesis care regime.
- **Eye irrigation should be only used for acid burns, petrol, detergents and alkaline burns.**
- When instilling more than one drop (unless otherwise indicated), a time lapse of at least five minutes must be allowed between each drop. This is necessary to allow the previous drop to be absorbed and not washed out by subsequent drop/s.² Level 5
- Eye ointments / gels are to be applied following eye drop installation, as they have extended absorption times.
- Check the eye for haemorrhage and discharge prior to administration of eye drops/ointment.

1.4 Procedural Information

1.4.1 Eye Hygiene – Eye Toilet

Requirements

- Eye toilet tray
- Sodium chloride 0.9% solution x 30ml for irrigation
- Eye pad (if required)
- Micropore (if required)
- Wet strength bag
- Tissues
- Personal Protective Equipment (Gloves)

Procedure

1. Perform hand hygiene.
2. Clean trolley using a combined detergent/disinfectant wipe.
3. Gather equipment. Perform hand hygiene.
4. Open eye toilet tray and prepare swabs using a non-touch technique.
5. Perform hand hygiene
6. Swab the operated or non-infected eye first using a non-touch technique, moving from inner to outer canthus. Use a clean swab each time.¹
7. Perform hand hygiene.
8. Continue until the eye is clean.
9. Instil eye drops or ointment as prescribed. Apply pad, shield or glasses as ordered.¹
10. Dispose of waste appropriately.
11. Perform hand hygiene.
12. Clean trolley using a combined detergent/disinfectant wipe.
13. Perform hand hygiene.

1.4.2 Eye Drops

Requirements

- Eye toilet tray
- Sodium chloride 0.9% solution x 30ml for irrigation
- Eye pad (if required)
- Micropore (if required)
- Wet strength bag
- Tissues

- Personal Protective Equipment (Gloves)

Procedure²

1. Perform 3 point ID check prior to administration of medication.
2. Explain procedure to patient including possible side effects of medications e.g. blurring of vision.
3. Perform hand hygiene.
4. Position patient in chair or bed and ensure adequate lighting.
5. Shake the bottle and re-check the drops with another nurse each shift.
6. Perform hand hygiene. Perform eye toilet if required.
7. Ask the patient to extend the neck and look up. Pull down the lower lid holding it firmly but gently against the underlying bone. Inspect the eye carefully for haemorrhage and discharge.^{2 Level 5}
8. Instil the drop in the centre of the conjunctival sac. Ask the patient to close eye gently and press firmly with finger against the inner corner of the eye lid (Punctal Pressure). Never drop directly onto the eyeball.^{2 Level 5}
9. Dab the eye, using a clean tissue following administration of eye drops.
10. Dispose of waste appropriately. Perform hand hygiene.

1.4.3 Eye Ointment

Requirements

- Eye toilet tray
- Sodium chloride 0.9% solution x 30ml for irrigation
- Eye pad (if required)
- Micropore (if required)
- Wet strength bag
- Tissues
- Personal Protective Equipment (Gloves)

Procedure

1. Explain procedure and possible side effects of medications to the patient
2. Perform 3 point ID check prior to administration of eye ointment/s.
3. Perform hand hygiene.
4. Position patient in chair or bed and ensure adequate lighting.
5. Perform hand hygiene .
6. Perform eye toilet if required.
7. Ask the patient to extend the neck and look up. Pull down the lower lid holding it firmly but gently against the underlying bone.^{2 Level 5}
8. Squeeze ointment (1cm) along lower border of conjunctival sac using a non-touch technique (take care not to touch the eye with the ointment nozzle). Do not rub lid to disperse ointment.^{2 Level 5}
9. Ask the patient to keep eye closed for a few seconds.
10. Perform hand hygiene.

1.4.4 Eye Irrigation

Requirements

- Eye toilet tray
- Sodium chloride 0.9% /Hartmann's solution 1LitreStandard IV administration set
- IV pole
- Absorbent sheet

- Kidney dish
- Tissues
- Wet strength bag
- Personal Protective Equipment
- Non Sterile Gloves
- Local anaesthetic drops as prescribed

Procedure³

1. Obtain consent from patient as appropriate. Perform hand hygiene.
2. Clean trolley using a combined detergent/disinfectant wipe
3. Perform 3 point ID check prior to medication administration. Perform hand hygiene.
4. Prepare prescribed irrigation fluid by connecting prescribed irrigation fluid to administration set (using a non-touch technique), prime line, connect fluid bag to IV pole and ensure easy access to patient.
5. Perform hand hygiene and don PPE including non-sterile gloves.
6. Protect the patient's clothing with an absorbent sheet.
7. Clean patient's eyelids and periorbital area.
8. Sit patient upright or lie with head tilted toward the side of the affected eye. Place the kidney dish against the patient's cheek on side to be treated. Ask patient to extend head and turn slightly to the side.
9. Instruct patient to keep both eyes open and to move eyeball around to ensure thorough irrigation. Gently open the eyelids with the index finger and thumb to allow thorough irrigation. Special care must be taken not to press the eyeball.³
10. Administer prescribed fluid at room temperature. Continuously irrigate the eye with irrigation solution gently from inner to outer canthus using a non-touch technique and ask the patient to move the eye in all directions.³
11. Continue irrigation until return flow is clear or as ordered.³ Avoid directing the stream forcefully over the eye ball
12. Avoid touching the eye structure.
13. Remove kidney dish. Dab eyelid dry with tissues and ensure the orbital area and cheek are dry.
14. Remove gloves and perform hand hygiene.
15. Instil eye drops using a non-touch technique.
16. Apply pad, shield or glasses as directed.
17. Discard waste appropriately. Perform hand hygiene.
18. Document and report any abnormalities noted during the procedure.
19. NOTE: Morgan lens may be used to irrigate eye. This is available from ED.

1.4.5 Removal of Contact Lenses⁴

Requirements

- Sterile gloves
- Contact lens storage case or 2 plastic specimen containers with lids marked (L) left contact lens and Right (R) contact lens
- Sodium Chloride 0.9% sterile sachet / Contact lens solution
- Patient identification labels
- PPE

Procedure⁴

1. Explain to the patient the need for removing contact lenses.
2. Ensure patient identification labels are on each contact lens container.
3. Where possible ask patient to remove the lens.

4. Perform hand hygiene.
5. Pour sterile sodium chloride 0.9% solution into storage containers.
6. Perform hand hygiene.
7. Don sterile gloves.

a) Removing soft lenses

- Open the eye using one hand – the index finger raising the upper eyelid and the thumb, pulling down the lower eyelid.
- Instruct the patient to look up.
- Use the index finger of the other hand to slide the contact lens down.
- Use the thumb and index finger to pinch the lens and lift it out of the eye.
- Repeat for the other lens.

b) Removing hard lenses

- Following procedure steps 1 to 5 above.
 - Place a thumb at each of the upper and lower eyelids.
 - Gently push the lids towards each other.
 - Cup your hand below the eye, to catch the lens.
 - Repeat for the other lens
8. Examine the contact lens for damage.
 9. Place the lens in the labelled container with solution.
 10. Examine the patient's eyes for redness and irritation.
 11. Remove gloves and perform hand hygiene.
 12. Document the removal of lenses in the inpatient notes.

1.4.6 Insertion And Removal Of Ophthalmic Prosthesis ⁴

Requirements

- Sterile dressing pack
- Sodium chloride 0.9% sterile sachet
- Eye ointment as prescribed
- Personal Protective Equipment, Non Sterile gloves

Procedure ⁴

Insertion:

1. Explain procedure and obtain patient's consent and co-operation as appropriate.
2. Perform 3 point ID check prior to administration of eye ointment.
3. Perform hand hygiene.
4. Perform eye toilet as necessary, refer to procedural information "1. Eye Toilet".
5. Perform hand hygiene.
6. Don non-sterile gloves.
7. Instruct patient to look down. Lift the upper lid up and insert the prosthesis into the fornix.
8. The lower eye lid is gently pulled down and forward allowing prosthesis to slip into the fornix of the eye socket.

Removal:

1. Perform hand hygiene.
2. Don non-sterile gloves.
3. To remove the prosthesis instruct patient to look up.
4. Evert the lower lid.
5. Place the index finger under the lower edge of the prosthesis.
6. Ease the prosthesis out with gentle pressure.

7. Place cupped hand/receptacle under the eye to catch the prosthesis.
8. Place prosthesis in container of warm water and wash with mild cleansing solution. All soap must be rinsed thoroughly from the prosthesis.
9. Rinse thoroughly.
10. Irrigate eye socket as required.
11. Remove gloves. Perform hand hygiene.
12. Document in the inpatient record

Related Documents

Legislation and Policy:

- Health Services Act 2016
- Medicines and Poisons Act 2014, Medicines and Poisons Regulations 2016
- Mandatory Policy MP0077/18 State-wide Medicines Formulary
- NMHS Clinical Documentation Policy
- SCGH Patient Identification and Procedure Matching Policy
- OPH Hospital Wide Policy Patient Identification
- SCGH Hospital Policy 141 Medicines Management

Key Related Documents:

- SCGH Nursing Practice Guideline Medication Management
- SCGOPHCG Infection Prevention and Control Manual, Aseptic Technique Policy
- SCGOPHCG Clinical Practice Guideline Falls Management

References

1. Dao Le LK. Eye Toilet [Evidence Based Recommended Practice on the Internet]. Adelaide: Joanna Briggs Institute; 2016 [updated 2016 Jan 8; cited 2017 Jan 11]. Available from: JBI Connect+ (Guideline)
JBI Levels of Evidence Level 5: Expert Opinion
2. Dao Le LK. Eye Medication: Administration [Evidence Based Recommended Practice on the Internet]. Adelaide: Joanna Briggs Institute; 2016 [updated 2016 Jan 8; cited 2017 Jan 11]. Available from: JBI Connect+ (Guideline)
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JBI Levels of Evidence Level 5: Expert Opinion
4. South Metropolitan Health Service. Eyes Management Clinical Practice Standard. 2013. (Guideline)

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